



Health Promotion Intervention Plan

NPU-T Community Maternal Care Network Taskforce

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Introduction

Maternal health is a major public health concern in Neighborhood Planning Unit–T (NPU-T) in Atlanta, especially when examining how consistent and timely prenatal services impact both maternal and infant outcomes. In this community, Black women and expecting families experience disproportionately higher risks of complications during pregnancy, including preterm birth and low birth weight. These disparities are not random. They are strongly connected to larger structural and social factors that affect access to care and overall health.

Recent data show that about 29.8% of births in NPU-T occur among individuals receiving inadequate prenatal care (Atlanta Regional Commission & Neighborhood Nexus, 2024). This is significant because prenatal care plays a critical role in identifying and managing complications early. When care is delayed or inconsistent, the risks for both the mother and infant increase. In fact, the community has a preterm birth rate of 13.1% and a low-birth-weight rate of 14.5%, which further highlights ongoing gaps in access to consistent, quality maternal healthcare (Atlanta Regional Commission & Neighborhood Nexus, 2024).

There are several factors that help explain these outcomes. Transportation is a major barrier, especially for individuals who do not have reliable access to a vehicle. Financial challenges, including gaps in insurance coverage, also make it harder to consistently access care. Beyond that, issues such as mistrust of the healthcare system and competing life demands can influence whether someone seeks care during pregnancy.

Altogether, these challenges point to a clear public health concern. Limited access to prenatal care is not just an individual issue. It contributes to broader patterns of maternal health inequities and poor birth outcomes. Understanding these conditions in NPU-T is important for

identifying what is driving these disparities and for informing strategies that can improve access to care and overall maternal and infant health outcomes.

Application of the Precede-Proceed Model

- A. The PRECEDE-PROCEED model, developed by Lawrence W. Green and Marshall W. Kreuter, is a widely used framework in public health that supports a structured approach to developing interventions based on identified community needs (Green & Kreuter, 2005). The model originated from the idea that effective public health interventions should not begin with solutions, but rather with a thorough understanding of the health issue and the underlying conditions shaping it. It was designed to help practitioners move beyond simply identifying a problem and instead examine the social, behavioral, environmental, and policy-related factors that contribute to health outcomes. Over time, the model has been widely adopted in community health and health education because it provides a structured and evidence-based approach to intervention planning. In the context of this intervention, the PRECEDE portion of the model is used to guide the development of a Maternal Health Access and Coordination Network in Neighborhood Planning Unit–T (NPU-T) in Atlanta. Specifically, Phases 1 through 5 are applied to examine how social conditions, health disparities, behavioral patterns, and environmental barriers contribute to limited access to prenatal care among pregnant Black women and expecting families in the community. By using this framework, the intervention is intentionally designed to address key determinants. These strategies include mobile prenatal clinics to reduce transportation barriers, as well as community-based educational efforts such as “Maternal Saturdays” and “Paternal Sundays,” which provide culturally

relevant prenatal education, health screenings, and support for both mothers and partners. Community health workers will play a key role in outreach and patient navigation by connecting individuals to services, providing reminders for appointments, and offering ongoing support throughout pregnancy and postpartum. Additional efforts will include partnerships with churches and community centers to host workshops, distribute health information, and create safe spaces for discussion, as well as transportation support through collaborations with local systems such as MARTA. The PROCEED phases will also focus on evaluating the effectiveness of these strategies by tracking outcomes such as increased prenatal care utilization and reductions in low birth weight and preterm birth rates.

B.

i. Phase 1: Social Assessment & Situational Analysis

Phase 1 of the PRECEDE-PROCEED model focuses on understanding how a health issue impacts the overall quality of life within a community (Green & Kreuter, 2005). In Neighborhood Planning Unit–T (NPU-T) in Atlanta, maternal health is closely connected to broader social and structural factors that affect daily life. Limited access to prenatal care is not just a healthcare issue, but is influenced by transportation barriers, gaps in insurance coverage, financial stress, and mistrust of the healthcare system. For pregnant Black women and expecting families in NPU-T, maternal health outcomes directly impact family stability and overall well-being. Approximately 29.8% of births in the community occur among individuals receiving inadequate prenatal care, which increases the risk of adverse outcomes such as preterm birth and low birth weight (Atlanta Regional

Commission & Neighborhood Nexus, 2024). These outcomes can lead to long-term health challenges for infants and increased stress for families, making maternal health a key factor in community quality of life. The community has a clear need for healthcare services that are accessible, affordable, and culturally responsive. Many residents would benefit from care delivered in trusted community settings, such as churches and community centers, where they feel more comfortable seeking support. In addition, there is a need to address practical barriers such as transportation, childcare responsibilities, and difficulty navigating the healthcare system.

ii. Phase 2: Epidemiological Assessment

Phase 2 of the PRECEDE-PROCEED model focuses on identifying the specific health issues that most strongly affect a community's quality of life by examining patterns such as prevalence, risk, and disparities (Green & Kreuter, 2005). In Neighborhood Planning Unit-T (NPU-T) in Atlanta, maternal health disparities are a key public health issue that significantly impacts both maternal and infant outcomes. One of the most concerning indicators is the high rate of inadequate prenatal care, with approximately 29.8% of births occurring among individuals who do not receive sufficient prenatal services (Atlanta Regional Commission & Neighborhood Nexus, 2024). Inadequate prenatal care increases the risk of pregnancy-related complications and limits opportunities for early detection and management of health concerns. In addition, the community experiences a preterm birth rate of 13.1% and a low birth weight rate of 14.5%, both of which

are associated with increased infant morbidity and long-term developmental challenges (Atlanta Regional Commission & Neighborhood Nexus, 2024). These health outcomes disproportionately affect Black women, who make up the majority of the population in NPU-T. This reflects broader patterns of health inequities, where structural and social factors contribute to unequal access to care and increased health risks. The trends observed in NPU-T highlight a clear need for targeted interventions to improve prenatal care utilization and reduce disparities in maternal and infant health outcomes.

iii. **Phase 3: Behavioral & Environmental Assessment**

Phase 3 of the PRECEDE-PROCEED model focuses on identifying the behavioral and environmental risk factors that contribute to the health issue (Green & Kreuter, 2005). In NPU-T, limited access to consistent prenatal care is influenced by a combination of individual behaviors and external conditions that make it difficult for pregnant women to engage in regular care. Behavioral factors include delayed initiation of prenatal care, inconsistent attendance at medical appointments, and limited knowledge about the importance of early and continuous prenatal services. Some individuals may rely on informal sources for health information or may not fully recognize the importance of attending regular prenatal visits, especially during the early stages of pregnancy. These behaviors can increase the risk of undetected complications and contribute to poorer maternal and infant outcomes. Environmental factors play a significant role in shaping these behaviors. Lack of reliable transportation remains a major challenge in NPU-T, particularly for individuals who do not have access to a reliable

vehicle. In addition, limited availability of nearby healthcare facilities may require residents to travel outside their immediate community for care. Financial barriers, including gaps in insurance coverage and high out-of-pocket costs, can further restrict access to services. Structural issues within the healthcare system, such as long wait times or limited appointment availability, may also discourage individuals from seeking or maintaining consistent care.

iv. Phase 4: Educational & Ecological Assessment

Phase 4 of the PRECEDE-PROCEED model focuses on identifying the factors that influence the behavioral and environmental risks outlined in Phase 3, specifically predisposing, enabling, and reinforcing factors (Green & Kreuter, 2005). These factors help explain why individuals engage in certain health behaviors and what conditions either support or prevent behavior change. Predisposing factors include individual knowledge, attitudes, and beliefs that influence decision-making. In NPU-T, some individuals may have limited awareness of the importance of early and consistent prenatal care, which can lead to delayed care initiation. This is reflected in the high rate of inadequate prenatal care in the community, where approximately 29.8% of births occur among individuals receiving insufficient care (Atlanta Regional Commission & Neighborhood Nexus, 2024). In addition, mistrust of the healthcare system, often shaped by past experiences or perceived discrimination, may discourage individuals from seeking care. Cultural beliefs and personal perceptions about pregnancy and healthcare can also influence how and when individuals engage with prenatal services. Enabling factors refer to the resources and conditions that

make it possible or difficult for individuals to access care. In this community, transportation barriers, limited access to nearby healthcare facilities, and gaps in insurance coverage all serve as barriers to prenatal care utilization. For example, approximately 28% of households in NPU-T lack access to a vehicle, which can make it difficult to attend regular prenatal appointments (Atlanta Regional Commission & Neighborhood Nexus, 2024). When services are not easily accessible or affordable, individuals may be less likely to attend appointments, even if they recognize the importance of care. On the other hand, the availability of community-based services, mobile clinics, and support from community health workers can act as enabling factors that improve access and engagement. Reinforcing factors include the social influences that encourage or discourage health behaviors over time. Support from family members, partners, and community networks can play a huge role in encouraging regular care engagement. Positive interactions with healthcare providers can also reinforce trust and increase the likelihood of continued engagement with care. In contrast, negative experiences or lack of support may discourage individuals from returning to care.

v. **Phase 5: Administrative & Policy Assessment**

Phase 5 of the PRECEDE-PROCEED model focuses on identifying the administrative and policy factors that may influence the implementation and long-term success of an intervention (Green & Kreuter, 2005). These factors include organizational capacity, resource availability, and existing policies that either

support or limit access to care. From an administrative perspective, several factors may affect the success of the Maternal Health Access and Coordination Network in NPU-T. One key consideration is the availability of funding to support mobile prenatal clinics, community outreach efforts, and educational programming.

Sustaining these services requires financial resources for staffing, equipment, and transportation. In addition, adequate staffing is essential, including healthcare providers, community health workers, and trained volunteers who can support outreach, patient navigation, and care coordination. Effective collaboration between healthcare systems, such as Grady Health System and Morehouse School of Medicine's H.E.A.L. Clinic, and community organizations will also be necessary to ensure that services are delivered efficiently and consistently.

External policy factors also play a critical role in shaping access to prenatal care. Medicaid coverage is particularly important, as many individuals in NPU-T rely on it to access prenatal services. Policies that expand or restrict Medicaid eligibility can directly impact whether pregnant women are able to receive timely care. In addition, local and state maternal health initiatives can support programs aimed at reducing disparities and improving outcomes. Transportation policies and public transit systems, such as MARTA, also influence access to care, particularly for individuals without reliable access to a vehicle. Given that approximately 28% of households in NPU-T lack access to a vehicle, transportation infrastructure plays a key role in determining whether individuals can attend appointments and access services (Atlanta Regional Commission & Neighborhood Nexus, 2024).

Stakeholder Engagement Plan

The key stakeholders for this intervention include Grady Memorial Hospital, Abraham's House Inc., 4Kira4Dads Paternal Center of Excellence, MARTA, Healthy Mothers Healthy Babies Coalition of Georgia, Morehouse School of Medicine, and the community members. Each stakeholder has an essential role in the development, implementation, and evaluation stages of this intervention plan that encompasses unique duties curated based upon their specific areas of expertise. The table below lists each key stakeholder, their roles in intervention planning, and how they will be engaged throughout this intervention.

Table 1. Stakeholder Assessment and Engagement Plan

Stakeholder	Roles in Intervention Planning	How and when to Engage
Grady Memorial Hospital	- Provides trained Community Health Workers (CHWs) to inform program design - Identifies community-level barriers to prenatal and postpartum care - Advise on outreach and enrollment strategies - Connect participants to prenatal care, benefits, and community resources	- Quarterly reports shared via email - Monthly virtual debriefings & budget planning (Zoom/Teams)

	• Deploy CHWs to identify and enroll pregnant residents during Paternal Saturdays & Sundays	
Abraham's House Inc.	• Serves as a trusted community partners to co-design culturally appropriate, family-centered programming in support of outreach strategies and efforts • Advise on aligning Paternal Sundays/Saturdays with existing community distribution schedules • Host Paternal Sundays/Saturdays with monthly food pantry distribution • Provides access food security support and resources (i.e. Bountiful Blessings food program)	• Promotional materials and flyers shared via email for all events • Text message blasts with updates monthly (frequency may vary) • Planning meetings and post event debriefings as needed (virtually or im-person)
4Kira4Dads Paternal Center of Excellence	• Advise on partner-focused maternal health education content	• Bi-monthly check-ins (evaluation of event attendance, effectiveness of

	• Support development of culturally responsive father and partner engagement strategies • Facilitate and plan Paternal Sunday/Saturday workshops • Provides 24/7 support access via AI chat to address any maternal health questions	workshops, community needs, and referrals) • Planning meetings and post event debriefings as needed (virtually or in-person)
MARTA	• Support development of transportation navigation resources (routes, passes, access points) for program participants • Oversee referrals and enrollment for transportation services • Provide free/ reduced-fare transit pass for program participants	• Quarterly reports shared via email • Monthly virtual debriefings & budget planning (Zoom/Teams)

Healthy Mothers, Healthy Babies Coalition of GA	• Advising on the selection of perinatal resources and educational materials most relevant to pregnant residents experiencing barriers to care • Co-hosting and planning monthly Community Baby Showers in NPU-T alongside local churches and program partners • Distribute perinatal care kits (safe sleep materials, breastfeeding supplies, maternal and infant health resources) • Leading and supporting maternal education workshops on pregnancy, postpartum care, and infant safety	• Promotional materials and flyers shared via email for all events • Text message blasts with updates monthly (frequency may vary) • Planning meetings and post event debriefings as needed (virtually or im-person)
Morehouse School of Medicine	• Advise on integrating maternal health-focused screening into existing community outreach models • Support development of culturally appropriate education and resources related to pregnancy-related risk factors	• Monthly check-ins (evaluation of screening events) • Virtual suggestion box 24/7

	(i.e. hypertension and gestational diabetes) • Collaborate with Community Health Workers and maternal health partners to develop prenatal screening protocols • Refer participants to appropriate CHWs and clinical partners for prenatal care • Provide maternal health screenings, educational materials, and rapid screening results at Paternal Sunday/Saturday events	accessibility through email • Education materials from local providers, CHWs, churches, and NPU-T taskforce for patient referrals
Community Members	• Co-designers of intervention: act as partners to identify priorities and goals tailored to the target population, while providing critical feedback throughout design process • Provide essential insight into community culture, values, and lived experiences • Trusted liaisons and advocates: leverage existing community relationships to bridge the gap between local organizations and residents, facilitating advocacy and information sharing	• Promotional and educational materials shared via street billboards, yard signs, church bulletins, email, NPU-T websites & social media accounts • Focus groups and in-depth interviews

		conducted twice a year • Monthly townhall meetings (both neighborhood townhalls & city/county meetings with city council members)
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Description of Intervention

Need

NPU-T faces persistent maternal health disparities among Black women that highlight the urgent need for this intervention. Between 2019 and 2024, NPU-T recorded 981 births with a preterm birth rate of 13.1%, a low birth weight rate of 14.5%, and 8.6% of births occurring to teens ages 15–19 (Atlanta Regional Commission, 2025). Additionally, 29.8% of births involved individuals who received inadequate prenatal care, far exceeding the national benchmarks (Atlanta Regional Commission, 2025). These outcomes are driven by overlapping behavioral and structural risk factors including delayed prenatal care initiation, limited nearby providers, transportation barriers, cost and insurance gaps, and longstanding healthcare mistrust rooted in

the historical mistreatment of Black patients (Musa et al., 2009). Without targeted, community-based intervention, these disparities are likely to persist across generations.

Context

The intervention will be implemented in community-based settings throughout NPU-T, including churches, community centers, and nonprofit hubs, with signature programming events hosted at trusted faith-based venues such as Abraham's House under Pathway Christian Ministries. Services are brought directly into the community to address the fact that 28% of NPU-T households lack vehicle access, with MARTA providing free or reduced-fare transit passes to enrolled participants (Atlanta Regional Commission, 2025). Additional environmental barriers, including provider shortages, insurance gaps, limited childcare, and healthcare mistrust, are addressed through Community Health Worker (CHW) mediated relationships, culturally responsive programming, and benefit enrollment support embedded within community events.

Target Population

The primary target population is pregnant Black women and expecting families ages 15–50 residing in NPU-T who experience barriers to obtaining consistent maternal healthcare. Based on 2020 U.S. Census data, approximately 5,450 African American women of childbearing age reside within the three Atlanta City Council districts (3, 4, and 10) overlapping NPU-T, providing the sampling frame for outreach and enrollment. A secondary population, including partners, fathers, and family members, is also engaged given research linking male partner involvement to improved maternal and infant outcomes (Musa et al., 2009).

Resources/Inputs

This intervention is supported by a range of human, institutional, and material inputs. Key personnel include CHWs working through Grady Health System, healthcare student volunteers from the MSM H.E.A.L. Clinic, and a program coordination team. Institutional partners contribute expertise, venue space, clinical services, educational materials, and transit support. Material inputs include prenatal education materials, perinatal care kits, MARTA transit passes, and mobile screening equipment, supplemented by external grant funding and in-kind contributions from partner organizations.

Activities

Core activities include monthly Maternal Saturdays and Paternal Sundays events offering on-site prenatal screenings, education workshops, benefit enrollment assistance, and food access. CHWs conduct continuous care navigation by enrolling participants, coordinating referrals to Grady Health System, and following families through the postpartum period. Monthly Community Baby Showers co-hosted with the Healthy Mothers, Healthy Babies Coalition of Georgia distribute perinatal care kits and serve as a low-barrier entry point. Community health data are collected through an adapted version of the Pregnancy Risk Assessment Monitoring System (PRAMS) surveys and qualitative interviews to continuously refine programming and referral pathways.

Outputs

Direct outputs include participants enrolled and screened, Maternal Saturdays and Paternal Sundays held, prenatal education workshops delivered, CHW referrals and follow-up contacts completed, MARTA transit passes distributed, Community Baby Showers hosted,

perinatal care kits distributed, and surveys and interviews collected. These outputs will be tracked through the program's shared data dashboard as indicators of implementation fidelity.

Outcomes

Short-term outcomes (1–2 years) include increased knowledge of prenatal resources, improved health literacy, increased healthcare trust, reduced transportation barriers, and improved partner engagement. Intermediate outcomes (3–4 years) include earlier prenatal care initiation, improved appointment attendance and care continuity, reduced structural barriers, improved help-seeking behaviors among pregnant women in NPU-T, and establishment of a sustainable care navigation infrastructure. Long-term outcomes (5+ years) include measurable reductions in NPU-T's rates of inadequate prenatal care (from 29.8%), preterm birth (from 13.1%), and low birth weight (from 14.5%), contributing to sustained improvement in maternal health equity for Black women in Southwest Atlanta. Figure 1 in the Appendix presents the logic model for this intervention.

Intervention/Implementation Team

To ensure the systematic execution and efficacy of the intervention, the project will be administered by a multidisciplinary team partitioned into specialized functional units. The Project Coordinator will serve as the central administrator, responsible for comprehensive project management, schedule adherence, resource allocation, and ensuring strict compliance with established ethical standards throughout all operational phases. Outreach and stakeholder engagement will be directed by the Community Impact Team, a coalition comprising local faith leaders, Neighborhood Planning Unit (NPU-T) representatives, and relevant municipal officials. This unit is tasked with establishing community trust, executing culturally competent recruitment

strategies, and mobilizing the target population to achieve an initial participation rate of 35% to 50%, with a targeted scale-up to 85% participation by the project's conclusion. Operating concurrently, the Data Team composed of selected municipal representatives and research professionals will manage the project's analytical framework. The Lead Researcher will maintain independent oversight of the data infrastructure to guarantee methodological integrity during survey administration, secure data collection, and the ongoing evaluation of key performance metrics. Finally, the Educational Team will oversee programmatic communication and knowledge dissemination. By developing and facilitating targeted workshops and community sessions, this team will clearly articulate the intervention's objectives, providing residents with actionable information and demonstrating how their participation directly contributes to measurable improvements within the community

Timeline

Intervention Plan and Timeline

The proposed intervention will be executed over a comprehensive two-year timeline, structured sequentially into three distinct phases. Phase 1, Planning serves as the foundational period prioritizing preparation and logistics. During this initial stage, the team will meet with community partners to align strategic goals, develop data collection tools and surveys, train staff and volunteers, secure requisite permissions from city council leaders, schedule key stakeholder events, and assign specific operational roles to all personnel. Phase 2, Recruitment, then transitions the focus toward active outreach and participant selection. Key activities in this phase involve mobilizing the community impact team to advertise the initiative, conducting baseline analyses to identify and select suitable candidates, and disseminating recruitment materials, including flyers,

social media toolkits, formational brochures, town halls and workshops. Ultimately, Phase 3, Implementation, represents the execution stage where the planned intervention is fully activated. Teams will fulfill their designated roles by hosting educational sessions and community workshops, administering tracking surveys to monitor progress, and conducting focus groups and interviews to gather the qualitative data necessary to thoroughly evaluate the program's overall impact.

Table 2. Timeline of Evaluation Activities

Activity	YEAR 1			
	Q1	Q2	Q3	Q4
Meet with community partners and stakeholders	x			
Develop surveys and data collection tools	X X X			
Train staff and volunteers on evaluation procedures				
Secure permissions and finalize implementation logistics				
Conduct baseline assessment / pre-intervention data collection		X X		
Advertise intervention and recruit participants				

Distribute recruitment materials and hold outreach events		X	X	
Administer tracking surveys during intervention			X	X
Conduct focus groups and interviews			X	X
Monitor participation and program progress			X	X
Analyze quantitative and qualitative data				X
Prepare evaluation summary/report				X

Intervention Budget

Table 3. Budget

Type of Expenditure	Amount Budgeted	Notes
Staffing		Personnel needed to support implementation of the intervention

A. Community health workers	\$60,000	Pay for community health workers providing outreach, education, participant support, and follow-up
B. Program coordinator	\$35,000	Oversees scheduling, communication, logistics, and intervention activities
C. Data collection/support staff	\$24,000	Assists with survey administration, record keeping, and participant communication
Materials and Supplies	\$	Items needed to support participants and intervention activities
A. Educational materials	\$3,000	Printed handouts, brochures, maternal health resources, and workshop materials
B. Food and refreshments	\$2,000	Meals, snacks, and drinks provided during intervention sessions or community events
C. Participant support supplies	\$5,000	Diapers, wipes, cleaning supplies, donation box materials, and other family support items
D. Media	\$4,000	Event Signage & Banners, Social Media Advertising, Community Publication Ads,

		Print Media & Outreach Materials
Equipment	\$	
A. Website building/operations	\$1,200	Website design, setup, maintenance, and online communication tools
B. Technology fee	\$2,000	Survey platform, software, internet support, or digital communication costs
C. Venue costs	\$5,000	Rental or use fees for churches, community centers, or meeting spaces
Travel	\$	
A. MARTA fare assistance/transit vouchers	\$4,500	Subsidized public transit support to reduce transportation barriers for participants.
B. Staff local travel reimbursement	\$2,500	Mileage, gas, or local transportation costs for staff/community health workers.
C. Parking/transportation support	\$1,000	Parking fees or other transportation-related expenses for events and outreach
TOTAL AMOUNT	\$ 145,200	

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Appendix

LOGIC MODEL

NPU-T Community Maternal Care Network | Goal: Improve maternal and infant health outcomes through increased access to quality prenatal and postpartum care

INPUTS "What we invest"	ACTIVITIES "What we do"	OUTPUTS "What we get"	SHORT-TERM OUTCOMES 1-2 Years	INTERMEDIATE OUTCOMES 3-4 Years	LONG-TERM OUTCOMES 5 Years +
- Funding & grants - Grady Health System - MSM H.E.A.L. Clinic - Healthy Mothers, Healthy Babies Coalition of GA 4Kira4Dads PCOE - Abraham's House / Pathway Christian Ministries - MARTA - Community Health Workers (CHWs) - Faith-based venues & community centers - NPU-T City Council (Districts 3, 4, 10) - Staff time & technical assistance	- Host Maternal Saturdays & Paternal Sundays - On-site prenatal screenings (BP, glucose) - Maternal & partner education workshops - CHW enrollment, referrals & follow-up - Distribute MARTA transit passes - Host Community Baby Showers - Distribute perinatal care kits - PRAMS surveys & qualitative interviews - Mobile care navigation & referrals - Faith & nonprofit partner outreach	- Participants enrolled & screened - Maternal Saturdays & Paternal Sundays held - Prenatal education workshops delivered - CHW referrals & follow-ups completed - MARTA transit passes distributed - Community Baby Showers hosted - Perinatal care kits distributed - Surveys & interviews collected - Clinical referrals & care navigation completed - Community partners engaged	- Increased prenatal care knowledge & awareness - Improved maternal & infant health literacy - Increased healthcare trust among Black women - Reduced transportation barriers - Partners & fathers better informed - Increased awareness of community services - Improved food security via Bountiful Blessings	- Earlier prenatal care initiation - Improved appointment attendance & continuity - Reduced cost, insurance & scheduling barriers - Strengthened community-clinical partnerships - Increased male partner engagement - Improved help-seeking behaviors - Sustainable care navigation established - Urban Maternity Health Fellowship at MSM launched	- Reduced inadequate prenatal care (from 29.8%) - Reduced preterm birth rate (from 13.1%) - Reduced low birth weight (from 14.5%) - Decreased maternal & infant mortality - Improved health equity for Black women in SW Atlanta - Sustained maternal health infrastructure in NPU-T

Figure 1. Logic Model

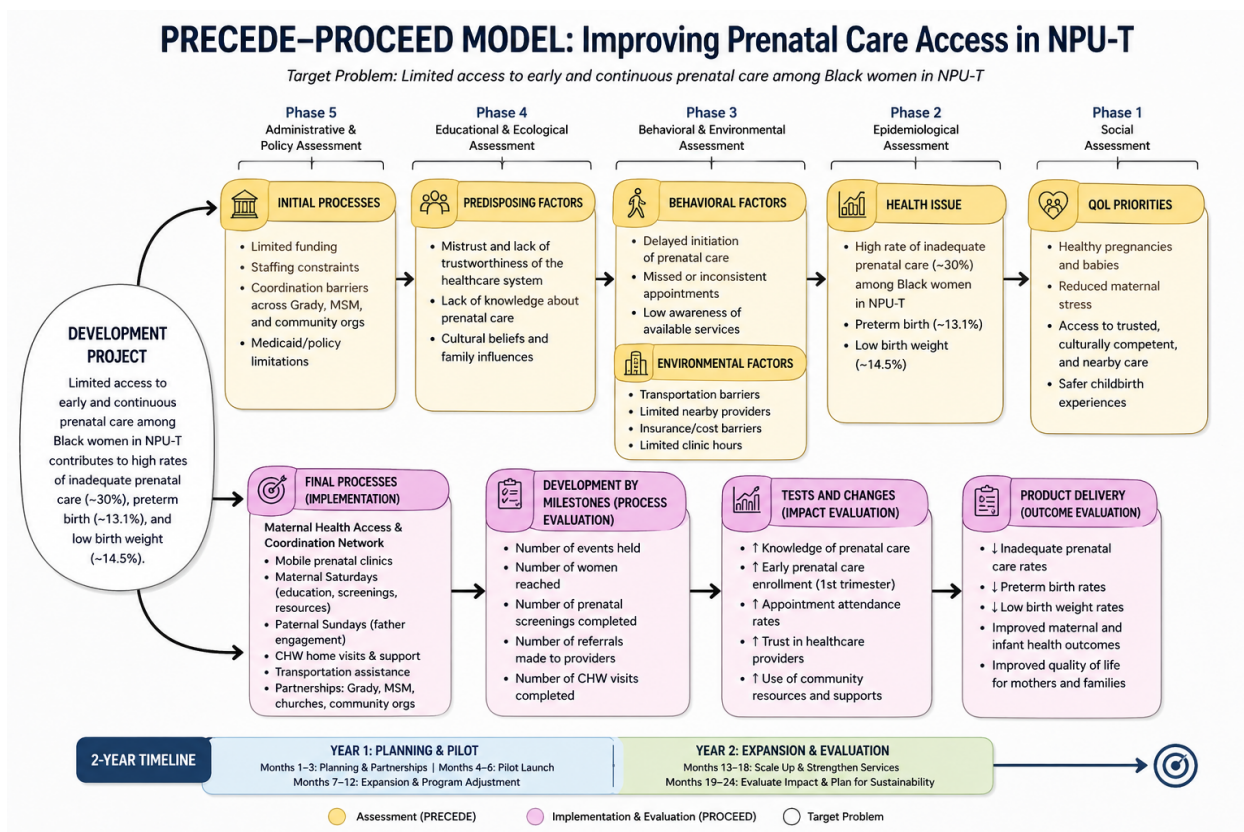


Figure 2. Precede-Proceed Model

Note. Adapted from ChatGPT (OpenAI, 2026).